SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

# Docetaxel 100 mg/m² · every 3 weeks

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Locally advanced or metastatic breast cancer · NCCP 00202 · version 8 · 21-day cycle

| Patient name | DOB | Hospital no. | Date |
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**Treatment context**
The aim is cancer control and symptom relief, with repeated review of benefit and tolerance.

**Schedule**
Docetaxel 100 mg/m² by IV infusion on Day 1 every 21 days, with dose changes when needed. The number of cycles follows benefit, tolerance and the agreed treatment plan.

## 1. GENERIC SACT RISKS / DISCUSSION POINTS

- ☐ Purpose, expected benefit and treatment plan explained
- ☐ Treatment may be delayed, modified or stopped because of toxicity
- ☐ Unexpected, rare or serious adverse effects can occur
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Pregnancy avoidance / contraception / fertility discussed where relevant
- ☐ Bone-marrow suppression / infection risk
- ☐ Anaemia / fatigue and low platelets / bleeding
- ☐ Nausea / vomiting and bowel disturbance
- ☐ Mouth soreness / mucositis and skin changes
- ☐ Allergic / infusion reaction and other organ-specific toxicity

## 2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

**Docetaxel**
TAXANE CHEMOTHERAPY

- ☐ Low blood counts, infection and sepsis
- ☐ Hair loss, mouth soreness and fatigue
- ☐ Numbness / tingling; sometimes lasting
- ☐ Fluid retention and nail changes
- ☐ Allergic infusion reaction

Discuss treatment aim, alternatives, uncertainty, fertility, contraception, access, phase changes, serious and lasting risks, supportive medicines and the urgent contact plan.

**Individual aim / benefit, alternatives, material risks and questions:**

## 3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

**Prototype only.** This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00202 v8.

SACTCheck v0.84.0
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